

Who Pays After Medicaid Expansion?

Cross-payer effects on health spending in US states, 2010–2018

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The question

Medicaid expansion raised Medicaid enrollment and Medicaid spending. That much is established across a large literature.

Much less is known about what happened to **everyone else’s** spending. If expansion pulled people out of private coverage, or changed what providers collect from other payers, it should show up in Medicare, private and out-of-pocket spending.

We estimate those cross-payer effects, and report how large a spillover these data could have detected.

Data

IHME Disease Expenditure Project (DEX 2.0): state-level spending and utilisation by payer and type of care, 2010–2018.

50 states and DC. Four payers: Medicaid, Medicare, private, out-of-pocket. Five settings: ambulatory, emergency, inpatient, nursing facility, pharmacy.

Enrollee counts and state population are recovered from the reported ratios, so coverage is measured inside the same source as spending.

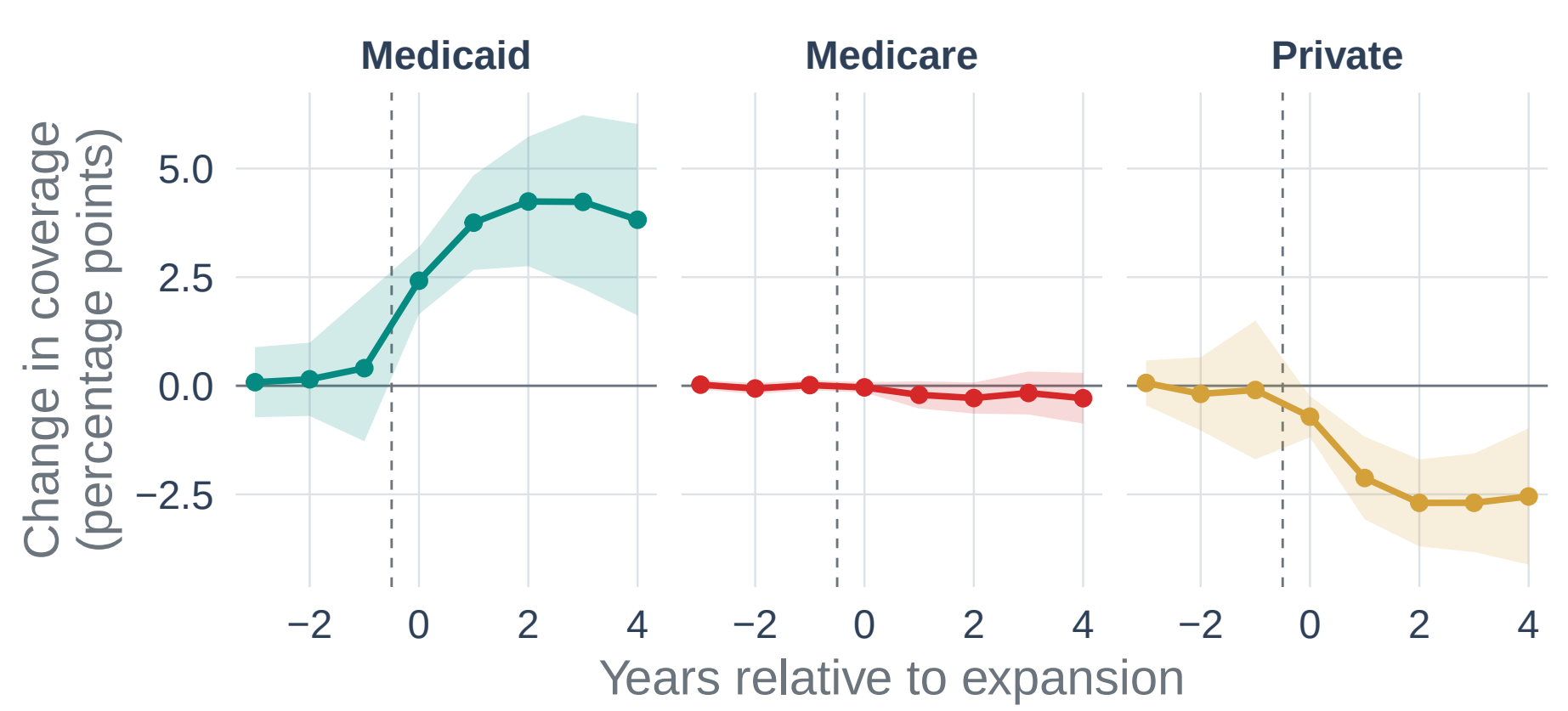
Design

Difference-in-differences comparing the **25 states that expanded on 1 January 2014** with the **19 that had not expanded by 2018**.

Callaway and Sant’Anna (2021) group-time estimator with never-expanded controls, adjusting for state poverty, median income, pre-expansion uninsurance and age structure. Standard errors clustered by state.

Later-expanding states are excluded. The 2015–2017 cohorts contain 3, 3 and 1 states, and at that size the estimator’s variance is not reliably identified.

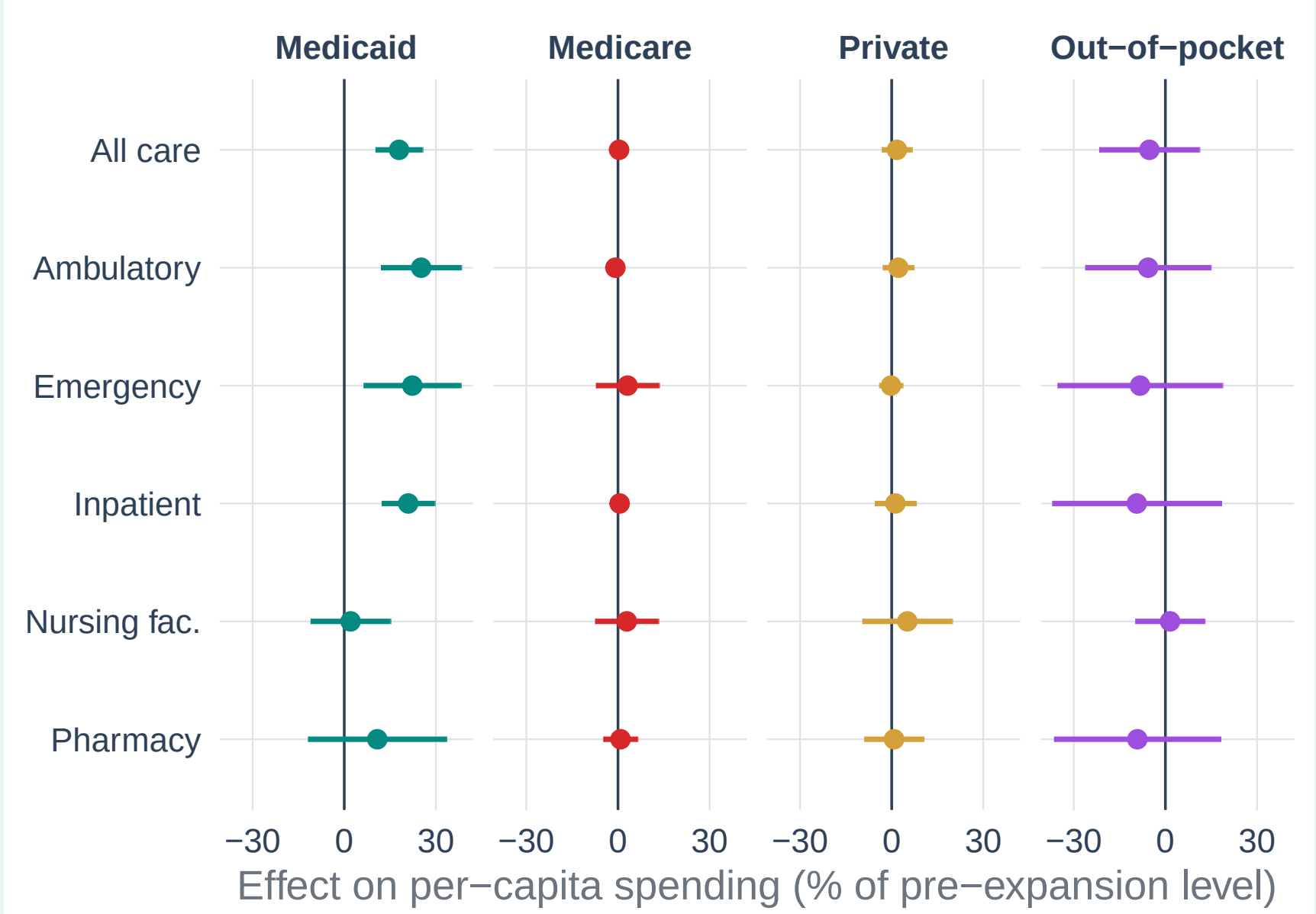
Expansion moved coverage



Medicaid coverage rises **3.7 points** (SE 0.8). Private coverage falls **2.2 points** (SE 0.4). Medicare does not move.

A strong first stage: a null on spending is not a null on treatment.

It did not move anyone else’s spending



Medicaid spending per capita rises **18%**. Across Medicare, private and out-of-pocket spending, in every one of the five settings, **no estimate is distinguishable from zero**.

What we can rule out

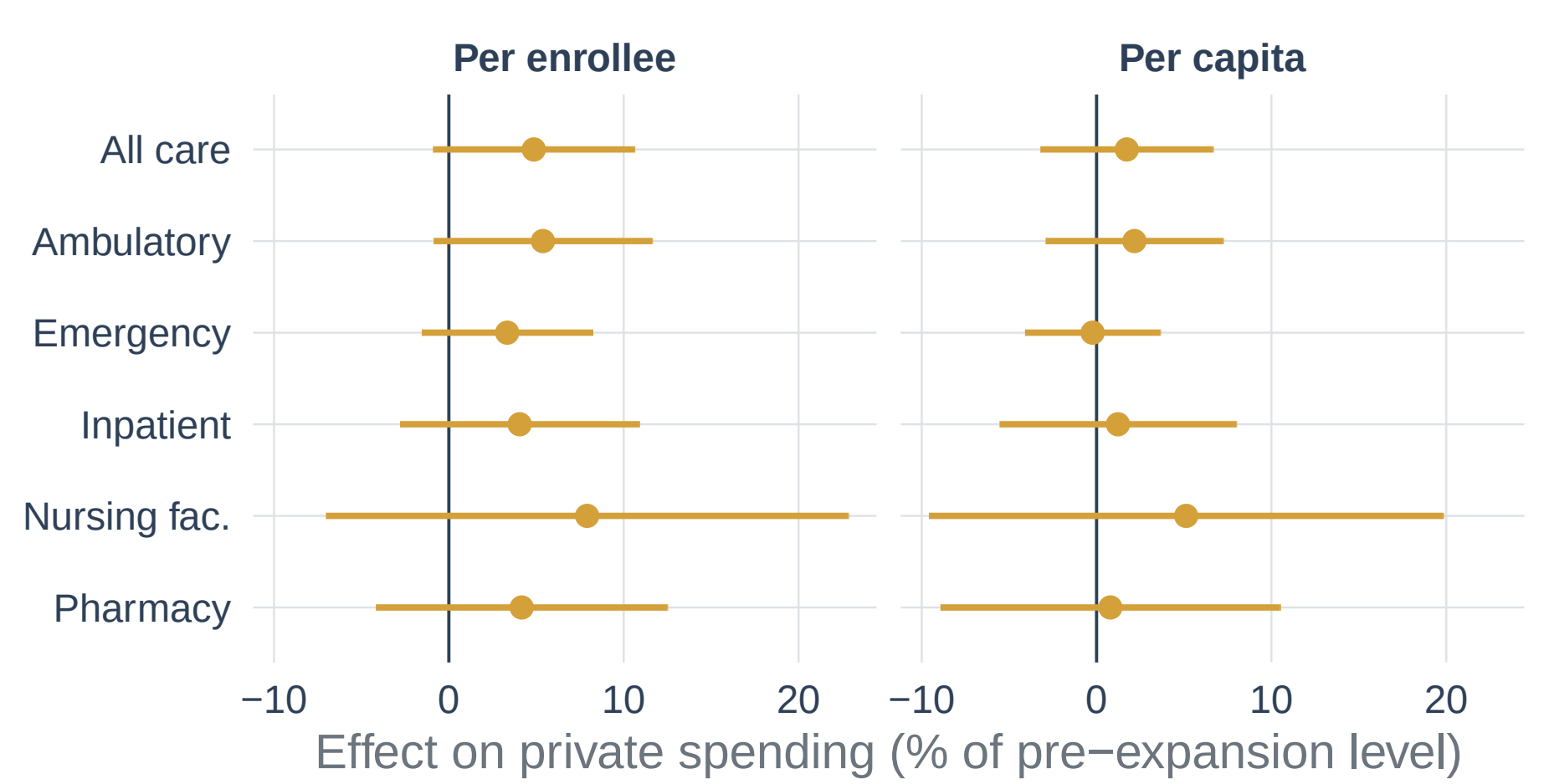
Effect on spending per capita, all care, as a share of the pre-expansion level:

Payer	Effect	95% CI
Medicaid	+17.9%	[+10.2, +25.6]
Medicare	+0.4%	[−2.2, +2.9]
Private	+1.7%	[−3.2, +6.7]
Out-of-pocket	−5.2%	[−21.7, +11.2]

The Medicare and private nulls are informative: spillovers beyond roughly **3%** and **7%** are inconsistent with these data.

The out-of-pocket null is not. Its interval spans a 22% fall to an 11% rise, so this design cannot speak to household spending.

Two patterns worth noting



Private spending **per enrollee** is positive in all six panels (+4.9% overall, CI [−0.9, +10.6]), while the same effect **per capita** sits at zero, because private coverage shrank. That is what compositional change looks like: lower-cost people moving from private coverage to Medicaid. Suggestive, not significant.

Emergency spending per enrollee also rises for both Medicare (+11.2, SE 8.6) and private (+3.7, SE 2.8) — the only outcome on which both non-Medicaid payers move together.

Limitations

Estimates describe 2014 expanders only.

Out-of-pocket has no enrollee denominator in DEX, so it is compared per capita only.

Medicaid price per unit fails the pre-trend test and is not interpreted.

Expansion moved coverage and Medicaid spending, and left Medicare and private spending where they were.

The absence of spillover is a result, not a gap: effects above 3% for Medicare and 7% for private are ruled out. For out-of-pocket spending, these data cannot yet say.